

SKIN & LASER

Burn, Blister & PIH Response

Immediate cooling, depth grading, blister management and the post-inflammatory hyperpigmentation pathway

DOCUMENT SKN-009	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO All clinical staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Every energy-device practice will eventually cause a thermal injury. This protocol defines the immediate response, how to grade what you are looking at, when to refer, and the structured pathway for post-inflammatory hyperpigmentation — the most common long-term consequence and the one patients find hardest to accept.

HOW YOU RESPOND IS WHAT IS REMEMBERED

Patients rarely sue over a complication that was recognized immediately, explained honestly, managed attentively and followed up daily. They very often pursue one that was minimized, dismissed, or discovered by them at home with no instructions.

2 Immediate Response

- 1

Stop treatment immediately. Do not finish the area, the pass or the session.
- 2

Cool the area with cool — not ice-cold — water or cool saline-soaked gauze for a sustained period. Cooling limits the depth of the injury and reduces pain.
- 3

Do not apply ice directly, and do not apply butter, ointment, occlusive balms, toothpaste or any home remedy to a fresh burn.
- 4

Assess and grade using the table below once cooling is underway.
- 5

Photograph immediately, at standardized angles, and at every subsequent review.
- 6

Tell the patient what has happened. Plainly, without minimizing and without speculating about blame.
- 7

Notify the medical director the same day, during the visit where possible.
- 8

Provide written instructions and same-day contact details before the patient leaves.
- 9

Arrange review within 24–48 hours. Do not leave follow-up to the patient to initiate.
- 10

Complete an incident report and record the parameters and circumstances while they are fresh.

DO NOT TREAT THE REST OF THE AREA

The instinct to finish so the result is symmetrical is strong and wrong. Asymmetry can be corrected later. A second burn cannot.

3 Burn Depth Grading

Depth	Findings	Action
Superficial	Erythema only, blanches with pressure, painful, skin intact, no blistering.	Cool compress, bland emollient, sun avoidance. Photograph. Review in 48 hours. Usually heals without sequelae.
Superficial partial thickness	Blistering, moist pink base, very painful, blanches with pressure.	Do not derroof intact blisters. Non-adherent dressing. Daily review. Medical director notified same day. Infection surveillance.
Deep partial thickness	Mottled red and white, reduced sensation, sluggish or absent blanching.	Same-day referral for medical assessment. High scarring risk. Do not attempt definitive in-office management.
Full thickness	White, waxy, leathery or charred. Painless. No blanching.	Immediate referral to emergency or a burn service. Do not delay for photographs or paperwork.

REFER IMMEDIATELY REGARDLESS OF DEPTH

Burns involving the eyelid or periorbital region, the lip vermillion, the nose, the genitalia, or a large surface area. Any circumferential burn. Any burn in a patient with diabetes, immunosuppression or impaired healing. Any burn where you are uncertain of the depth.

4 Blister & Wound Management

Situation	Approach
Small intact blister	Leave intact. The blister roof is the best available biological dressing and protects against infection. Cover with a non-adherent dressing.
Large or tense blister in a mobile area	Prescriber decision. If drainage is indicated, it is performed aseptically by authorized personnel, leaving the roof in place as a dressing.
Already ruptured blister	Gentle cleansing, non-adherent dressing, infection surveillance. Do not debride in-office beyond loose necrotic material.
Crusting	Leave to separate naturally. No picking, no scrubbing, no exfoliation.
Signs of infection	Increasing rather than settling pain, spreading erythema, purulence, odour, fever or malaise. Same-day medical assessment; culture where discharge is present.

- ☐ Daily review — in person or by dated photograph — until re-epithelialized.
- ☐ Written wound care instructions supplied and explained, with a copy in the chart.
- ☐ Strict sun avoidance from the outset; PIH risk begins immediately.
- ☐ Antibiotic or antiviral cover only on prescriber decision, documented.
- ☐ No further energy-device treatment to the area until fully healed and reviewed.
- ☐ Referral to dermatology or plastic surgery for any deep injury, delayed healing or developing scar.

- ☐ Scar management plan discussed once the wound has closed.

5 Post-Inflammatory Hyperpigmentation

PIH follows inflammation of any cause and is substantially more common and more persistent in Fitzpatrick III–VI. It typically appears within weeks and can take many months to resolve. Patients need that timeline stated early and repeated, because the gap between expectation and reality is where the relationship breaks down.

- 1 **Photograph and document** the affected areas at standardized angles.
- 2 **Sun protection is the foundation** and is non-negotiable. Broad-spectrum, daily, reapplied, plus physical protection. Any ultraviolet exposure prolongs and deepens PIH, and no topical treatment will outrun it.
- 3 **Restore the barrier first.** Bland cleanser and emollient. Do not start actives on inflamed or broken skin — that worsens the pigmentation.
- 4 **Introduce pigment-directed therapy** once the skin is intact, per the medical director's authorized regimen. Tyrosinase inhibitors and retinoids are commonly used; your medical director specifies the agents, sequence and duration.
- 5 **Introduce one agent at a time** and reassess. Stacking actives on recently injured skin causes irritation, and irritation causes more pigment.
- 6 **Review at defined intervals** with photographs from the same angles.
- 7 **Set the timeline explicitly:** months, not weeks, and improvement is gradual.
- 8 **Defer any further energy-based treatment** to the area until the PIH has fully resolved and the medical director has reviewed.

DO NOT TREAT PIH WITH MORE ENERGY

The instinct to laser away the pigment you caused is a serious error. Energy delivered to recently inflamed, pigmented skin usually deepens the problem and can make it permanent. Topical management and time, under review, is the pathway.

HYPOPIGMENTATION

Loss of pigment is less common but far more difficult. It may be delayed by months and may be permanent. Photograph, notify the medical director, refer for dermatological opinion, and do not attempt to treat it with further energy.

6 Documentation & Disclosure

- Device, all parameters, passes and total energy delivered at the index treatment.
- Skin type recorded, test patch history, and screening completed at that visit.
- Time of recognition and the immediate actions taken, with times.
- Serial standardized photographs from identical angles.
- Grading applied at each review and the reasoning.

- Medical director notification time and the guidance received.
- Every patient communication, including what was said and what was provided in writing.
- Referrals made and their outcomes.
- Incident report and the root cause review — what allowed this, and what has changed.
- Manufacturer reporting where a device fault is suspected.
- Malpractice carrier notified in line with policy terms.

NEVER ALTER THE RECORD

Do not retrospectively amend parameters, screening entries or notes after a complication. Systems log it, it is generally discovered, and it converts a defensible clinical outcome into an indefensible credibility problem. Corrections are made by dated addendum only, per OPS-002.

7 Quick Reference

QUICK REFERENCE CARD

Suspected burn

1. **Stop. Do not finish the area.**
2. **Cool water or cool saline gauze**, sustained. No ice, no ointments.
3. **Grade it.** Blistering = at least superficial partial thickness.
4. **Photograph now** and at every review.
5. **Leave intact blisters alone.** The roof is the dressing.
6. **Eyelid, lip, nose, large area, or uncertain depth → refer today.**
7. **Tell the patient plainly. Notify the medical director same day.**
8. **Review in 24–48 hours.** Never leave follow-up to the patient.
9. **PIH: sun protection first. Never treat it with more energy.**

Print, laminate and post at the point of care

8 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

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